

POSC 3710: European Politics

9/23/2026: CASE STUDY — HEALTH CARE SYSTEMS

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What should a national welfare state look like

- Suppose it fell to you to design a national health system from scratch
- Who would be eligible for care? Everyone, or only some people?
- What would it cost the patient at the point of care?
 - Nothing?
 - Capped costs?
 - Market rate?
- How would it be funded: taxes, premiums, employers, or some mix?
- And how would you pay the doctors?
 - How much would they get paid?

Why Health Care Is Hard

- Along with pensions, it is the largest social expenditure in almost every rich country
- The moral case: most illness arrives by chance
 - Health determines life chances
- The economic case: demand is inelastic. Nobody shops around while having a heart attack
- The investment case: a healthy, treated workforce is a more productive one
- Every rich democracy grapples with healthcare provision. The solutions have been quite varied

What do we need to decide on?

COVERAGE

Who is in?

- Everyone as a right, everyone by mandate, or only those who qualify
- Universal coverage is the European norm; the United States is the outlier
- Coverage on paper is not the same as access in practice

FUNDING

Who pays?

- General taxation, earmarked payroll contributions, or individual premiums
- Taxation pools risk across everybody; premiums pool it across the insured
- Whoever pays is who gets angry when costs rise

PRICE AT USE

What does the patient pay?

- Free at the point of use, a flat fee, or a deductible and co-pays
- Charging at the door deters unnecessary visits — and necessary ones
- This is the choice patients notice first

PROVIDERS

How are doctors paid?

- Salaried state employees, or independents billing fee-for-service
- This is the one doctors fight hardest over
- Remember this on the second half of today

The United States, Britain and the Netherlands, side by side

	UNITED STATES	BRITAIN	NETHERLANDS
Coverage	No universal coverage; employer plans, Medicare, Medicaid, and the uninsured	Universal, automatic, free at the point of use	Universal by mandate: every resident must buy a basic policy
Funding	Mixed: premiums, employers, federal and state programs, and out of pocket	General taxation + National Insurance contributions (think US payroll tax)	Payroll contributions and taxes, plus the premium you pay yourself
Who provides	Private hospitals and physicians; insurers in between	Hospitals are state-owned and staff are NHS employees	Private insurers and private providers, tightly regulated
What patients pay	Deductibles, co-pays and surprise bills; cost is the leading worry	Nothing at the door. Prescription charges in England only	A yearly deductible, then covered. Low earners get a subsidy
The trade-off	Fast, advanced care for the well-insured; nothing guaranteed for the rest	Nobody is excluded, but you may wait	Choice and universality, at the price of a compulsory purchase

Britain: The NHS

- Founded in 1948 by Aneurin Bevan, Clement Attlee's health minister, out of the Beveridge report
- Free at the point of care for everyone, funded from general taxation
- Hospitals are publicly owned and the doctors and nurses are public employees
 - US average doctor salary: ~ \$375,000. UK Average ~ \$115,000
 - UK junior/trainee doctors start around \$40,000
- It is the closest thing in Europe to a genuinely socialized health service
- It is also the most popular institution in Britain
 - Also probably the most complained about

In all systems, somebody has to say no

- A system that is free (or even just affordable) cannot say yes to all treatments. Rationing is necessary
 - How do we ration in the US?
- Britain rations openly. NICE, a national body, decides what the NHS will pay for
- Its tool is the quality-adjusted life year: one year in full health is one QALY
 - Roughly, a treatment buying a year of good health for under £20,000–30,000 clears the bar
- Britain is unusually explicit in rationing, but any public or private insurance-based model is also rationing.
 - Medicare and Medicaid ration too.

The NHS: Under Strain

- The waiting list for treatment is long and has grown in recent years
 - Particularly long to see specialists
- About 7.1 million treatments were waiting in March 2026, down from a record 7.7 million in 2023
- More than a third of emergency patients waited over four hours that month
- Lord Darzi's 2024 investigation found a service damaged by underinvestment and reorganization
- Nobody proposes to abolish the NHS
 - Still hugely popular with voters, desire to invest more

The Dutch Model: Regulated Competition

- Everyone must buy insurance.
 - Insurers must sell to anyone, at one price, regardless of health
- The government defines the basic package
 - Insurers compete on price and service, not on who they cover
- Premiums run about €160 a month, with a yearly deductible of €385 and a subsidy for low earners
- Do not insure yourself and you are fined, enrolled anyway, and billed from your wages
- Satisfaction and access are both high

How the Dutch Got There

- Before 2006 the Netherlands had two tiers: about two-thirds in a public scheme, the rest on the market
- Private coverage was expensive and the middle of the income distribution struggled with it
 - Sound familiar?
- The 2006 reform was passed by a center-right government, not a left one
- It is close to what the Affordable Care Act was originally meant to be
 - Attacked from the right as too much government intervention and the left as not generous enough/not free

The United States Model

- Five Different Systems
 - employer insurance (most common)
 - Medicare if over 65
 - Medicaid (138% of the federal poverty line if in expansion state)
 - Individual coverage via state exchanges
 - Being uninsured
- For the well-insured, the care is excellent
 - My care is *very good*. I personally would probably be worse off under most European systems
- It is also the only rich democracy where medical bills are a normal cause of bankruptcy
- In Europe, healthcare coverage is seen as a right. That is contentious in the US

What It Costs

- The United States spends 18% of its GDP on health care
 - ~Half of this is government spending
- The comparison countries, 8-12%
- US spends more overall, and same amount via government
- American life expectancy runs more than four years below the ten-country average
 - US has the highest rates of preventable and treatable death in the group
- Are there any justifications

Overall health system performance, ten high-income countries

Ranks out of ten; 1 = best

Country	Overall	Access to care	Care process	Administrative efficiency	Equity	Health outcomes
Australia	1	9	5	2	1	1
Netherlands	2	1	3	6	3	7
United Kingdom	3	2	8	1	5	8
New Zealand	4	5	1	3	8	3
France	5	6	7	4	6	5
Sweden	6	4	10	7	—	6
Canada	7	7	4	5	7	4
Switzerland	8	8	6	10	4	2
Germany	9	3	9	8	2	9
United States	10	10	2	9	9	10

So Why Are They Different?

- Three rich democracies, each with different answers to how to provide healthcare
- The easy explanation is that doctors everywhere resist being put on a salary
 - That is true.
- Medical associations fought national health insurance in every country
 - But they won in some places and lost in others. The doctors cannot be the whole story
- Immergut has another explanation

Veto Points, Not Veto Groups

- If medical professionals could block policy by themselves, no country would have public healthcare
- If so, no country would have socialized medicine
- Her claim: the blocking occurs in the political system
- A veto point is a place in the process where an organized opponent can stop a bill
- What are some examples of veto points?

Where opponents of national health insurance could stop a bill

	SWITZERLAND	FRANCE	SWEDEN
Veto Points	In the electorate: any law can be challenged by popular referendum	In parliament: shifting coalitions and short-lived governments	In the executive: a stable governing majority that could legislate
What opponents did	Took national insurance to the voters and defeated it	Worked through deputies to soften each bill	Lobbied and lost
Policy Result	No compulsory national insurance until the 1990s	National health insurance: but doctors kept fee-for-service	A tax-funded service with salaried hospital doctors

Why no Public Healthcare in the US?

- Single member districts make coalition hard
- Federalism
- Senate filibuster
- Most Americans under 65 get coverage through an employer
 - Cost is there but hidden
 - Most people like their coverage
- Policy feedback: a program creates the constituency that then defends it
 - A big push against public option during Obamacare debates; claims that you wouldn't be able to keep the same doctor, government rationing etc

Discussion

- What challenges does the NHS face in the next decade? What does the Dutch system face?
- What do these two do better than the American system, and what do they do worse?
- If Immergut is right, what would have to change here before a different system was possible?
- Which country would you prefer to be sick in? How do you determine?

The Federal Republic of Clemson

- Key things to think about:
 - who is covered
 - who pays
 - what it costs at point of service
 - how providers are paid
- “Good health care for all” is not a position!